# EPIDEMIOLOGY OF RESUSCITATION
## KEY MESSAGES

GUIDELINES 2025
EUROPEAN RESUSCITATION COUNCIL©

### Establish national registries

All European countries should have comprehensive national registries for Out-of-Hospital Cardiac Arrest (OHCA) and In-Hospital Cardiac Arrest (IHCA) according to the Utstein template

### Use multidisciplinary teams for counselling

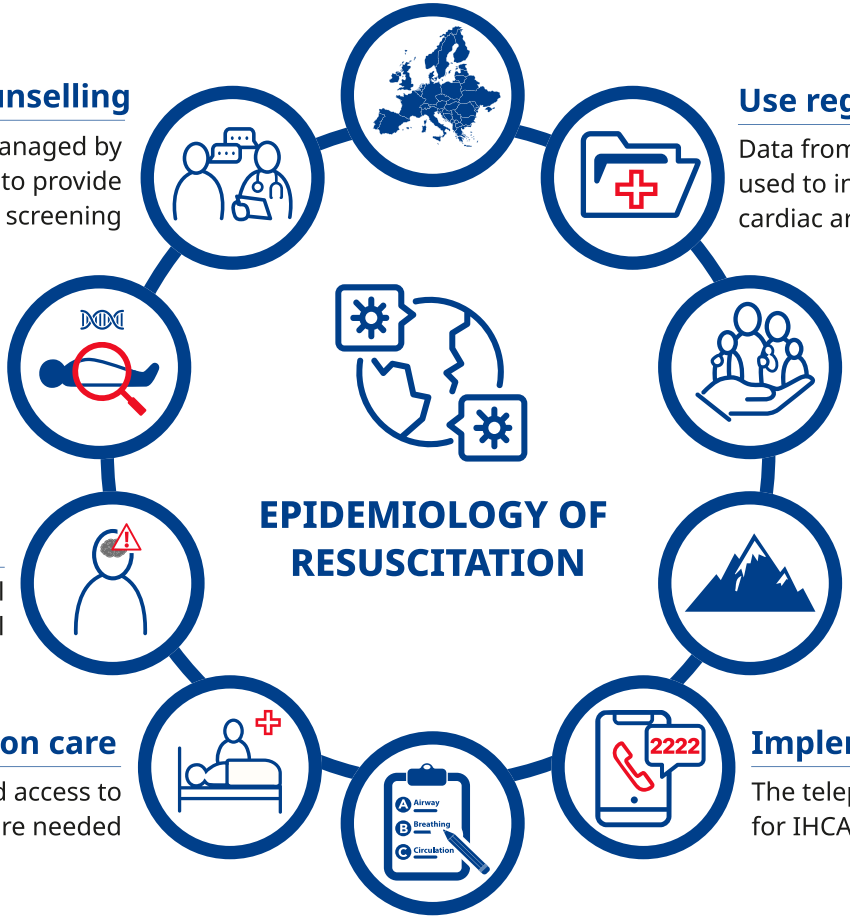
Autopsy and genetic results should be managed by multidisciplinary teams in specialised clinics to provide family counselling and eventual screening

### Use registry data for system planning

Data from OHCA and IHCA registries should be used to inform healthcare system planning and cardiac arrest responses

### Conduct comprehensive autopsies

All victims of unexpected sudden death under age 50 should receive a full autopsy, including genetic analysis using 5-10 ml of blood in EDTA




### Support low-resource settings

Epidemiological registries must be developed in low-resource settings to allow improvement of treatment and outcomes

### Measure long-term patient outcomes

Routine measurement of physical and non-physical outcomes for all cardiac arrest survivors is essential

### Improve response systems in remote areas

Improved emergency response systems must be developed in remote areas to improve outcomes

### Enhance post-resuscitation care

More research and expanded access to post-resuscitation rehabilitation services are needed

### Implement 2222 for IHCA

The telephone number 2222 should be standardised for IHCA response across Europe

### Expand IHCA research

There is a need for increased research efforts focused on in-hospital cardiac arrest in Europe

IHCA - in-hospital cardiac arrest; OHCA - out-of-hospital cardiac arrest